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The real-time experiences of older people with dementia, informal caregivers and professional caregivers during the transition from home to a nursing home: multiple longitudinal cases

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Abstract

Background Retrospective research has shown that the transition from home to a nursing home is a complex and often fragmented care process, which can lead to negative outcomes for the entire care triad: older people with dementia, informal caregivers, and professional caregivers. At present, comprehensive and real-time research covering the entire transitional care process is lacking in the literature. This study aimed to follow the experiences of the care triad during the transition from home to a nursing home in real time.

Methods This real-time, multiple-case study employed the patient journey mapping methodology. Participants were recruited in the province of Limburg, the Netherlands, through partner healthcare organisations affiliated with the Living Lab in Ageing and Long-Term Care. Five care triads were included, each consisting of an older person with dementia awaiting nursing home placement, their informal caregivers, and the professional caregivers involved in the transition process. A total of 26 participants were interviewed. Data were collected between October 2021 and March 2023 through semi-structured interviews, follow-up moments, and detailed logbooks. Data analysis involved both individual and cross-case analysis.

Results Care triads experienced an overarching sense of uncontrollability throughout the transition process. This was linked to three key factors. First, dependency within the care triad was marked by each participant feeling reliant on others. Second, transitions were crisis-driven rather than planned, as participants struggled to identify the right timing. This was influenced by uncertainty, societal taboos, and government policy. Third, the transition process was largely discontinued, with different professionals responsible for separate parts of the process and the dementia case manager becoming absent after the move.

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Conclusion Policymakers and professional caregivers should collaborate to establish a more gradual transition process. Professional caregivers should adopt a guiding role and ensure continuity of care after the move. Future research should explore how to empower care triads throughout the transition from home to a nursing home.

Keywords Transition, Dementia, Nursing home, Real-time research, Patient journey

Background

Older people with dementia generally prefer to remain at home for as long as possible. However, a move to a nursing home may become necessary when the home situation is no longer sustainable. In such cases, nursing homes can offer a safe living environment where care is provided by professional caregivers [1, 2]. The transition from home to a nursing home is a complex and emotionally demanding care process for everyone involved. This process engages what is often referred to as the care triad: the older person with dementia, their informal caregivers, and the professional caregivers supporting the transition. The transition unfolds across three phases [3, 4]. In the pre-transition phase, a decision is made regarding the move, and a suitable nursing home is selected [5–8]. The mid-transition phase begins with placement on a waiting list and ends on the actual day of the move [7–9]. Finally, the post-transition phase centres on adjustment to and acceptance of the new living environment [5, 7–9].

The transition from home to a nursing home is a care process that engages multiple theoretical constructs. According to Schlossberg's transition theory, a transition is a (non-)event that results in change. However, it is only considered a transition when the individual involved experiences that change. The impact of the transition is shaped by four factors: the situation (e.g., the timing of the transition), the self (e.g., personal commitment), support (e.g., from family), and strategies (e.g., coping responses) [10]. Schlossberg's theory suggests that a fragmented and uncoordinated care process—particularly one marked by crisis and lack of communication with the older person—can result in adverse outcomes for all parties involved [10]. Specifically, older people with dementia may experience a sense of separation from their community and loved ones [11]. The transition can also be linked to a decline in physical and/or mental health and a reduced quality of life [11–13]. Informal caregivers may experience guilt, frustration, or even depression [14], while professional caregivers can be burdened by the duplication of services [15]. To enhance this care process, a model developed by Groenvynck et al. (2021) identifies four key needs for older people and informal caregivers: access to clear information, effective communication, sufficient emotional and practical support, and adequate time. These elements are critical for fostering a partnership with healthcare professionals throughout the transition, which is often emotionally demanding

[4]. However, most existing studies and models on transitional care are retrospective [16], meaning that data were collected only after the transition process had concluded.

Retrospective data collection is subject to recollection bias. Participants may struggle to accurately recall past events [16], particularly when describing emotionally charged experiences such as the move to a nursing home [17]. For example, Boots et al. (2015) found that while early-stage dementia caregivers often had difficulty accepting professional help at the time, they later reported wishing they had received early, personalised support to aid the acceptance process [18]. These findings illustrate how retrospective accounts of needs and experiences may differ significantly from those captured in real time. Similar discrepancies have been observed in research on the transitional care process. Groenvynck et al. (2022) reported that informal caregivers, in retrospect, expressed a preference for a timely and well-coordinated transition. However, in real time, these same caregivers often delayed the decision to move the older person into a nursing home until the home situation became unsafe [19]. Capturing experiences as they unfold—especially during emotionally significant events such as care transitions—is therefore essential.

Real-time data collection enables the identification of challenges and gaps within the care process and offers valuable insight into the shared decision-making involved [20, 21]. One method used to explore real-time experiences is patient journey mapping. This methodology investigates individuals' experiences and perspectives across various healthcare settings and contexts. It helps to uncover shortcomings within the healthcare system while also providing a unique view of the care triad's experience [22]. However, real-time, longitudinal data collection is rarely undertaken because of its time-intensive nature [23]. As a result, research capturing the real-time experiences of the care triad during transitional care remains largely absent from the literature.

Further knowledge is needed to understand how the real-time experiences of the care triad emerge during the transitional care process, as well as what the implications are for the quality and organisation of care. Consequently, this study aims to answer the following research question: *“How do older persons with dementia, informal caregivers, and professional caregivers experience the transition from home to a nursing home in real time?”*

Methods

Design

This study employs the patient journey mapping methodology [22], which captures a representation of a person's experiences and emotions throughout a care journey [23]. This approach is intensive and time-consuming [23]. Data were collected between October 2021 and March 2023.

Context

This study was conducted in Limburg, a province in the southern Netherlands. In the Dutch healthcare system, older adults diagnosed with dementia are supported by a dementia case manager. This is an independent professional assigned to support both the person living with dementia and their informal caregivers. Dementia case managers typically hold a degree in nursing or social work and have completed post-bachelor training. They provide information, guidance, and support to help navigate the healthcare system. However, only 33–41% of people with dementia currently receive support from a case manager [24]. The involvement of the case manager ends once the older person moves into a nursing home.

At present, nursing home care in the Netherlands is primarily funded by the government, provided that individuals contribute financially based on their personal assets [25]. In theory, older persons and their informal caregivers are free to choose a nursing home that aligns with the older person's needs and preferences, without incurring additional costs, regardless of the care setting. However, this financially favourable system has an important caveat: older people with dementia are only eligible to move into a nursing home if they require 24-hour supervision and support. Furthermore, the limited availability of nursing home beds in the Netherlands has resulted in extensive waiting lists. These long waiting times make the timing of the move unpredictable and greatly restrict the ability to choose a suitable facility. This can increase the burden on both older people and informal caregivers, complicating the transitional care process [26].

Participants and recruitment strategy

The study aimed to include care triads, each comprising at least one older person with dementia, an informal caregiver, and a professional caregiver. The target was to recruit between four and six care triads.

A two-step recruitment approach was employed. First, dementia case managers who were willing to participate and who were supporting an older person meeting the inclusion criteria were recruited. These case managers were approached through information sessions held at partner healthcare organisations of the Living Lab in Ageing and Long-Term Care in the Netherlands.

The potential professional caregivers then asked their clients—older people and their informal caregivers—whether they were interested in taking part in the study. Information brochures were provided to offer further details. If the older people and their informal caregivers expressed interest, the researcher contacted them to schedule a home visit. During this visit, the researcher explained the study's purpose and procedures. The home visit also allowed the researcher to become acquainted with the participants and to arrange a follow-up visit to begin data collection. Informed consent was obtained during the first visit. Additional informal and professional caregivers involved in the transition were recruited over the course of the study.

All care triads were selected based on a predetermined list of inclusion and exclusion criteria. Participants had to be 65 years of age or older, have a formal diagnosis of dementia, and be on the waiting list of a preferred nursing home. Their informal caregivers were required to provide some form of care for the older person and to be both able and willing to participate in the study. Professional caregivers had to be involved in the care of the older person with dementia and actively engaged in the transitional care journey.

Data collection

Data were collected between October 2021 and March 2023. This study employed three data collection methods: semi-structured interviews, follow-up moments, and detailed logbooks. By using these multiple methods, the researchers aimed to capture the care triad's prospective, real-time, and retrospective experiences throughout the transition process. Data collection began once the older person, informal caregiver(s), and professional caregivers had signed the informed consent. It continued until a few months after the move, the older person's death, or the termination of the study. All data were collected by the lead researcher, who holds a nursing degree.

Semi-structured interviews

Each interview began with the collection of demographic information. The first interview was conducted with the dementia case manager to gather background details and specific information about the participants (e.g., whether the older person had a sensory impairment). The number of interviews varied depending on participants' roles in the transition process. When the older person was unable to participate, proxy interviews were conducted. The interviews were both event-specific and participant-specific. Event-specific meant that interviews were conducted before and/or after a transitional care event (e.g., the day of the move). Participant-specific indicated that each interview was tailored to the individual, and therefore no single, standardised interview guide could be

used. Interview guides were developed based on previous conversations with the interviewees and the specific care event at the time (e.g., the week prior to the move). Interviews were conducted in person, by telephone, or via a secure internet connection, depending on the participants' preferences. The focus was on capturing participants' experiences throughout the transition, including their views on the organisation of care and their experience of the care provided. In addition, some generalised questions and topics were informed by the TRANSCIT model [4], which emphasises the importance of communication, information, support, time, and partnership throughout the transition process (General topic guide: Appendix 1). Two examples of personalised topic guides are provided in Appendix 2. All interviews were audio recorded.

Follow-up moments

The participants were contacted regularly by the researcher, either by telephone or in person. These follow-up moments provided an opportunity for participants to reflect on their expectations and experiences between the transitional care events. They also enabled the researcher to stay informed about upcoming events in the care trajectory. In addition, follow-up conversations helped identify other stakeholders involved in the process (e.g., a care coordinator at the nursing home who recently joined the care team). These stakeholders were recruited over the course of the study.

Logbook

A logbook was maintained for each case, containing summaries of the follow-up moments and interviews. The researcher recorded observations, reflections, and experiences encountered during the study. This approach allowed for a thick description of the context and supported a nuanced, case-specific report of each care triad.

Data analysis

The data were analysed by three independent researchers. The lead researcher (LG) analysed all data, while two co-authors (AB and EdV) each analysed half of the dataset. The analysis began with content analysis to develop an in-depth understanding of each individual case. This process started with the transcription of individual interviews [27].

Next, the researchers applied a cross-case analysis methodology, aiming to generate more generalisable findings and to identify the conditions under which these findings occurred [28, 29]. The analysis focused on understanding the experiences of the care triad and the factors influencing those experiences.

Data triangulation, using the logbooks, was employed to verify the findings and enrich them with additional

contextual detail. This approach enabled the creation of a comprehensive overview of the transitional care experiences of the different participants within each case.

Reflexivity

The researcher immersed herself in the lives of the older people and their families by regularly visiting and calling participants for up to 1 year. She was aware that such ongoing involvement could influence participants' experiences of the transition process [30]. To address this, all participants were asked how the interviews and visits had affected their thinking. These reflections were considered during the analysis. To support a more objective interpretation of the findings, multiple researchers were involved in the data analysis. This approach reduced the risk of emotional moments during interviews unduly influencing the results. In addition, both data and investigator triangulation were employed to enhance the credibility of the study's findings [31].

Ethics

In accordance with the Declaration of Helsinki, ethical approval was obtained from the Medical Ethics Committee of Zuyderland. The committee confirmed that the provisions of the Netherlands' Medical Research Involving Human Subjects Act did not apply (registration number METCZ20210044).

Results

Participants

Six care triads were recruited, of which five ultimately participated. One triad withdrew from the study because the older person moved to a nursing home shortly after recruitment. At that point, the primary caregiver felt that participation would be too burdensome for the older person.

Across the 5 cases, 26 participants were interviewed. A total of 71 interviews and follow-up moments were conducted—an average of 14 per case. Table 1 provides an overview of the participants' characteristics. The interviews lasted an average of 51 min, with durations ranging from 16 to 107 min. Data collection for each case study spanned between 6 and 12 months (see Table 2). All but one older person (Case 4) moved to a nursing home during the study period. Table 3 presents an overview of the transitional care journey for each case.

Themes

Throughout the transition from home to a nursing home, all individuals within the care triads experienced an overarching sense of lost control. There was no such thing as an 'optimal' transition. This overall feeling of uncontrollability stemmed from an experienced dependency within the care triad, a crisis-driven rather than a planned

Table 1 Participant characteristics

Participant role		Age (years)	Sex	Interviews and follow-up moments
Case 1				
Older person (husband)	· Older persons were a couple. Both were diagnosed with dementia.	92	Male	3
Older person (wife)	· The husband had moderate dementia. o His wife had advanced dementia. She could not participate in the study. · They received home care to aid with activities of daily living and household chores.	86	Female	She did not participate.
Informal caregiver (daughter)	· The primary family caregiver was the daughter. She and her husband provided support and care. · The daughter also had a brother who was not involved during the transition process.	57	Female	6
Dementia case manager	· The dementia case manager was involved at the start of the study. · The professional caregivers of the nursing home declined participation.	62	Female	5
Case 2				
Older person (husband)	· The older person was a husband. He was diagnosed with dementia. · He was not involved during the study because he was not informed of the transition decision. · He did not receive care at home.	82	Male	He did not participate.
Informal caregiver (wife)	· The primary caregiver was the wife of the older person. · They lived together before the move. · They had three children (one daughter and two sons).	77	Female	10
Daughter	· She was involved, but she resided in Belgium and therefore had to travel 2 hours to visit her parents.	48	Female	1
Son	· He lived close by and was available whenever his parents needed him.	52	Male	1
Dementia case manager	· The dementia case manager was involved from the moment of diagnosis.	55	Female	6
HCP nursing home (intake conversation)	· She performed the intake interview with the informal caregiver and the family on the day of the move.	27	Female	1
HCP nursing home (contact person)	· She was the contact person for the family. However, the family still had not spoken with her when the data collection was stopped.	Unknown	Female	1
HCP (waiting list coordinator)	· She was an administrative coordinator of the transitional care process.	37	Female	1
Case 3				
Older person (husband)	· The older person was a husband who had progressive supranuclear palsy and dementia. · He initially experienced mostly physical symptoms. · He mostly had mobility problems and was at risk of falling.	74	Male	5
Informal caregiver (wife)	· The informal caregiver was the wife. They lived together. · They also had a son and daughter who supported their parents throughout the transition process.	71	Female	6
Daughter	· The daughter lived farther away but was always available when her parents needed her.	48	Female	1
Son	· The son lived closer and often joined the meetings with the dementia case manager.	46	Male	1
Dementia case manager	· The dementia case manager had a leading role. She prepared the family for the transition.	44	Female	2
HCP (day care)	· She was the coordinator of the day care centre that the older person visited before moving to a nursing home.	59	Female	1
HCP (waiting list coordinator)	· The waiting list coordinator was personally involved in the transition process. She was in contact with the family and conducted regular phone calls to provide information about the preferred nursing home or their status on the waiting list.	37	Female	2
HCP nursing home (contact person)	· She was the contact person for the family from the moment the older person began living in the nursing home.	30	Female	1

Table 1 (continued)

Participant role		Age (years)	Sex	Interviews and follow-up moments
Case 4				
Older person (husband)	· The older person was a husband. · He still lived at home with his wife at the end of the study. · He had dementia and was informed about the transition decision. He had significant physical health problems. · He received home care and went to a day care facility. · He did not participate in the study because his wife preferred not to involve him.	84	Male	He did not participate.
Informal caregiver (wife)	· The primary informal caregiver was the wife of the older person. · She also had physical health problems.	76	Female	4
Dementia case manager	· The dementia case manager provided care to the older person and his family.	50	Female	2
HCP home care (care coordinator)	· The home care coordinator helped the older person with activities of daily living.	61	Female	2
Case 5				
Older person (mother)	· The older person was a mother. · She had advanced dementia and was no longer able to communicate. · She resided in the first nursing home until 6 months before the study. She lived back at home for another 6 months. · She moved to a new nursing home at the start of the study.	86	Female	She did not participate.
Informal caregiver (daughter)	· The informal caregivers care were two sisters. · They also had two brothers who were less involved.	59	Female	3
Informal caregiver (daughter)	· They removed the mother from the first nursing home because of dissatisfaction with her care.	56	Female	3
Dementia case manager	· The dementia case manager provided care in the home situation.	41	Female	1
HCP (waiting list coordinator)	· The nursing home coordinator provided an overview of the older person's daily life and care at the nursing home.	Unknown	Female	2

transition, and a largely fragmented and discontinued transition process (see Fig. 1).

Older people and informal caregivers, together with the dementia case manager, initially established a clear transitional care plan. They had registered on the waiting list of a preferred nursing home, and some had even visited the future residence. However, these carefully constructed plans were disrupted by real-world challenges, including long waiting times, difficulties in maintaining consistent support from dementia case managers, and fragmented care provision. As a result, older people and informal caregivers felt powerless and dependent on the decisions of professional caregivers and the constraints of the healthcare system.

Dependency within the care triad

Throughout the transition process, there was a continuous sense of dependency among the members of the care triad. Older persons—often not actively involved in the transition—felt reliant on both informal and professional caregivers. Informal caregivers, who felt responsible for the older person's care and wellbeing, in turn depended on the approach and support of professional caregivers. The decision to move and the timing of the transition were also shaped by the older person's health status and the caregiving capacity of the informal caregiver. Finally,

professional caregivers were themselves dependent on the decisions and preferences of older people and their informal caregivers.

The older persons

At the beginning of the transition, most older individuals (Cases 1, 3, and 4) were actively involved in the process and had a voice in the decision regarding the move. They were invited to participate in formal conversations alongside professional caregivers. Informal caregivers highlighted the importance of open and honest communication, as well as the need to share information transparently with the older person.

Informal caregiver (wife), case 3: Together, we do everything together. We have discussed the entire trajectory from the beginning to the very end.

The involvement of older individuals in the transitional care process declined as their cognitive abilities deteriorated. Although there was initially a strong intention to keep them actively engaged throughout the transition, this commitment was not maintained as their cognitive health worsened. This shift can be attributed to two main factors. First, both family members and professional caregivers were concerned that the transition might be

Table 2 Overview of data collection moments

	Oct 2021	Nov 21	Dec 21	Jan 22		Feb 22	Mar 22	Apr 22	May 22	Jun 22	Jul 22	Aug 22	Sep 22	Oct 22	Nov 22	Dec 22	Jan 23	Feb 23	Mar 23			
Case 1	Mid			Post			Weeks after move														Post	
20 Oct 21–25 Jan 2022 n=11 contact moments	D - D	D- DIO - OI	I - I			I - D	D		D - IO												I	
Case 2		Mid		Post			Weeks after move															
9 Nov 21–29 Apr 22 n=25 contact moments		D- D - I - DI - D	I - D - I - I - I - D	D - HN - I		D - F - F	HN - DI - I - HN	D - I - I - I														
Case 3									Mid			Post		Weeks after move								
5 Apr 22–21 Mar 23 n=11 contact moments									D - D	IO	IO	IO - IO - I	D - D - HH	HN- HN	IO		F-F		HN	I		
Case 4									Mid													
7 Apr 22–24 Nov 22 n=8 contact moments									HH	HH	D	I		I	I - D					I		
Case 5							Mid	Post	Weeks after move													
23 Mar 22– 16 Sep 22 n=8 contact moments							D - D	D		I	IC - HN		I - HN									

Legend			
Pre-transition			
Mid-transition			
Post-transition			
Weeks after the move			
F	Family	O	Older person
D	Dementia case manager	HH	Healthcare professional (homecare)
I	Primary informal caregiver	HN	Healthcare professional (nursing home)
	Time between conversations	X	Moved to a nursing home

too distressing for the older person. They feared that disclosing the details of the move could provoke resistance. Second, neither professional nor informal caregivers had sufficient information about the specifics of the move. This uncertainty made it difficult to sustain open communication. As a result, in three cases (Cases 2, 3, and 5), families constructed a false narrative to ease the transition. Once the older person was settled in the nursing home, they were informed that this would be their permanent place of residence.

Informal caregiver (daughter), Case 3: So, initially, we said, to get him: 'We are going to a place where they are going to look how to help you better. It is temporary...' So we haven't been completely honest. My mom said if we tell him about the move, he will

want to know when and what. We don't know that either.

Once the older persons had moved into the nursing home, they became dependent on the nursing home staff. At this stage, dementia case managers were no longer able to advocate on their behalf. This sense of dependency was intensified by the older persons' physical and cognitive decline, which was particularly evident in Case 1.

Informal caregiver (daughter) and older person (husband), Case 1: Daughter: You don't like it here. Older person: Really not, I cannot stand it anymore. I already say this when I wake up in the morning. I am still getting out of bed, but I do not like it. I don't know what to do. Life here is disappointing. Daughter: Yes, you cannot be yourself. You are living at the mercy of others.

Table 3 Overview of the transition process per case

		Phone call, placed at preferred nursing home	Day of the move	End of study or death of participant	
		Time on the waiting list for the preferred nursing home	The week before the move and the day of the move	First weeks after the move	Months after the move
Case 1	Older person	• The couple had a preferred nursing home and were offered a double room. • They only wanted to move if they could live together. • Both were involved in the transition decision.	• The husband was overwhelmed by the news. • The wife was no longer able to understand the transition decision; she was always happy. • The move occurred 1 week after the phone call • At the nursing home, they got a double room so they could live together, but it was on a somatic ward.	• The husband significantly declined after the move and quickly became wheelchair bound. • He was not happy at the nursing home. • The wife became incontinent; she seemed happy and could still participate in activities.	• The husband wanted to move back home; he did not feel at home in the nursing home. • At home, he would be alone. • He wanted to die, asked for and received palliative sedation, and no longer wished to live. • The wife died a few days after the husband; cause unknown.
	Informal caregiver	• The daughter felt the waiting time was too long due to administrative tasks. • She lost her freedom and needed to care for her parents 24/7.	• News of the move fell sudden. • The daughter was doubtful about the move and considered keeping them at home longer.	• She visited every day. • She was exhausted, and the transition had a significant impact on her life. • Decorating the room was not allowed because of restrictions in the nursing home.	• She was dissatisfied with the care: mouldy food, dirty clothes, missed appointments. • She continued questioning the possibility of returning home.
	Professional caregivers	• The dementia case manager visited every 6 weeks, arranged the transition, and fixed small problems until the move. • She said she would be present until 6 weeks after the move.	• Due to miscommunication between professional caregivers, information was not always passed to the older persons and the informal caregiver.	• The dementia case manager was no longer involved. • Communication with nursing home staff was challenging because of constant staff changes. • The staff were not used to caring for people with dementia.	• Staff and informal caregivers communicated by writing notes on paper. • Staff were barely present.
Case 2	Older person	• The older person was not interviewed because he did not know about the transition decision.	• He moved to a preferred nursing home 5 minutes from his home. • He attended day care and was accompanied to the nursing home.	• He was shocked on the day of the move. • He did not resist the family's decision. • He became very sick shortly after the move.	• His physical and mental health deteriorated significantly. • He experienced some agitation.
	Informal caregiver	• The transition felt paradoxical; the wife understood the need for it but did not want to let go. • She planned the transition ahead of time. • She felt her role shifted from wife to caregiver.	• She doubted the decision; the phone call came earlier than expected. • She began preparing for the move. • She called for a 'family moment' the week before the move.	• She was not involved on the day of the move and was anxious about the older person's reaction. • The daughter and son accompanied the older person on moving day. • The wife felt satisfied only if she visited him daily.	• She recognised she was burned out from caregiving. • She was satisfied with the timing of the move. • She did not visit him every day. • She felt she had regained her freedom and hobbies.
	Professional caregivers	• The case manager was proactive. • She initiated advance care planning and communicated with other caregivers.	• There was a miscommunication with the nursing home staff.	• The case manager provided information about the nursing home. • She was not involved on the day of the move because the older person had a strong support network.	• Nursing home staff communicated with the family through an online system. • They provided structure that was lacking at home.
				person had a strong support network.	• Staff were friendly and experienced.
Case 3	Older person	• The older person was involved in the transition decision. • He was on the waiting list for a preferred nursing home. • He had mostly physical problems and a speech impediment. • He received home care and attended day care.	• A crisis occurred after his health deteriorated. • He was first moved to an observation centre, then to an available nursing home. • He was not informed about the move because of the urgency and uncertainty of the situation.	• He had difficulty accepting the new living situation. • He blamed the informal caregiver and accused her of leaving him behind. • His mental and physical health declined significantly. • He became wheelchair bound.	• He deteriorated rapidly within a few days. • He died 6 months after moving to the nursing home.
	Informal caregiver	• The wife was the primary caregiver. • Because of the high fall risk, she had to supervise him constantly and experienced a high care burden. • The preferred nursing home was located across the street.	• She was no longer able to care for him because he began exhibiting incomprehensible behaviour. • She and the children accompanied him to the observation centre. • They received a call and had to move him within 2 hours.	• She visited the nursing home every day. • It was a 30-minute drive, leaving her with little time for herself. • She felt emotional because of his incomprehensible behaviour.	• She was grateful that he died without suffering. • They had a loving farewell. • She said the transition was harder than his actual death.
	Professional caregivers	• The case manager proactively prepared the family for the move. • The older person received support from home care nurses.	• The dementia case manager arranged the crisis transfer. • The care coordinator remained in contact in case a place became available at the preferred home.	• The staff tried their best to care for him. • A significant staff shortage increased his fall risk.	• The nursing home coordinator ensured he died peacefully. • Staff attended the funeral to pay their respects.
		• The care coordinator of the preferred nursing home kept in touch with the family and answered questions.		• The family often helped care for other residents when staff were unavailable.	
Case 4	Older person	• The older person was not involved in the study. • He was informed about the transition decision. • He was present during discussions with the dementia case manager, but it was unclear whether he understood everything because he often lost interest during the meetings. • He attended day care four times a week.			
	Informal caregiver	• The wife was the primary informal caregiver. • She also attended a support group for caregivers of people with dementia every 2 weeks. • She found it difficult to care for him, especially having to constantly tell him what he could or could not do. • She did not feel ready for the nursing home transition and needed to feel it was the 'right' time. • The son was involved and attended all meetings with the dementia case manager.			
	Professional caregivers	• The dementia case manager recognised the caregiver burden. • She attempted to accelerate the transition process out of concern that it could become a crisis. • She provided information about care options but felt the informal caregiver needed to make her own decision, even if the case manager disagreed with it.			
Case 5	Older person	• The older person moved into a nursing home near her daughters. • She was no longer able to communicate and could not participate in the study.		• She had previously requested euthanasia, but the request was denied because she appeared happy despite her diagnosis.	
	Informal caregiver	• The transition felt very sudden. • They had been waiting 6 months and received a phone call requiring a move within 2 days. • The nursing home allowed time for adjustment, and the informal caregivers continued providing care. • One daughter worked in healthcare and had knowledge about dementia.		• The daughters initiated a process to request euthanasia for their mother, which was denied. They were disappointed by the decision. • A new resident caused significant noise on the ward. The daughters began to doubt whether the nursing home was the right choice.	
	Professional caregivers	• The case manager sent information about the older person's care to the nursing home. • She wanted to call the nursing home herself because the case was unique. • She explained expectation management to the daughters, noting that it would not be one-on-one care like at home. • The nursing home staff also had to manage the daughters' expectations.		• Communication with the daughters occurred via electronic records. • There were frequent tensions because the daughters paid close attention to every detail of their mother's care. • The staff and the daughters did not always agree. The staff perceived the daughters as controlling. The daughters felt they were simply trying to do what was best for their mother.	

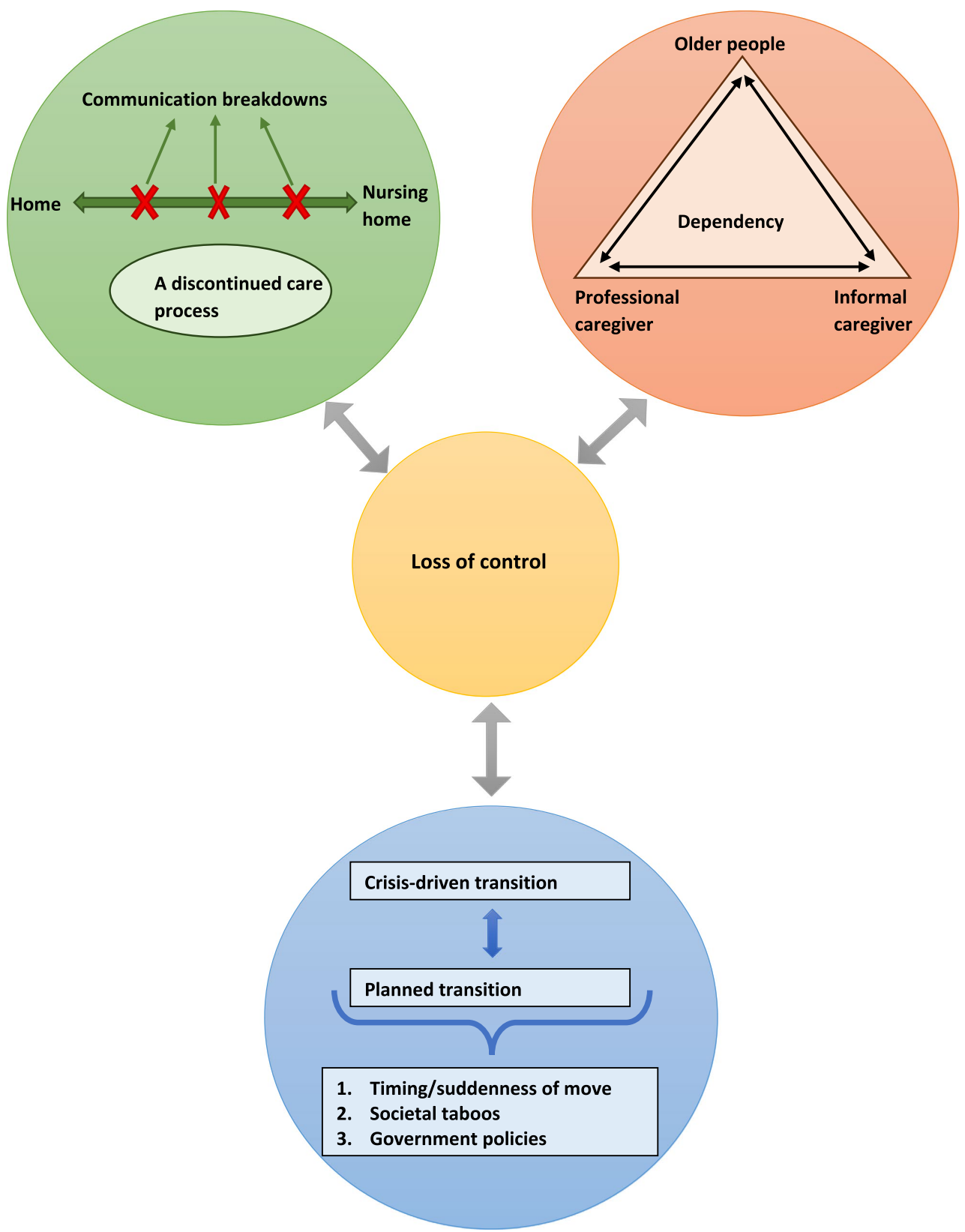


Fig. 1 Care experiences of older people, informal caregivers, and professional caregivers during the transition from home to a nursing home

The informal caregivers

At the outset, while the person with dementia was still living at home, informal caregivers took on the role of full-time carers. This dependency of the older person on their informal caregiver was often accompanied by negative emotions. Caregivers felt burdened by the many responsibilities and experienced a loss of personal freedom. This is illustrated in the following quote, in which the wife of an older person describes how her role shifted from being a spouse to becoming a full-time caregiver.

Informal caregiver (wife), Case 2: I went into the hallway myself and said: 'Come on and turn on the light.' He then comes. These are difficult moments for me. It is like a child that needs guidance.

Moreover, informal caregivers were tasked with making the difficult and emotional decision to initiate the transition. This responsibility was particularly burdensome for adult children acting as primary caregivers. By contrast, spouse caregivers often received support from their children, who recognised the challenges and emotional strain their parent was facing. In these situations, the children often took on the role of mediator in the care process, signalling when additional support from professional caregivers was needed. They shared in the responsibility of the transition decision, easing the burden on the spouse caregiver.

Informal caregiver (daughter), Case 3: The family told my mother: 'If this continues, two people will end up in a nursing home because you cannot keep doing this.'

This situation was different for primary child caregivers. Although they often had siblings, the responsibility for making the transition decision typically fell to them alone. The other children in the family often felt that because they were not directly involved in providing care, they were not entitled to a final say in the decision. Moreover, their limited involvement in the daily care of the older person meant they often lacked a clear understanding of the care burden experienced by the primary caregiver.

Informal caregiver (daughter), case 1: We have doubts, should we do it? Can't we leave them at home? However, it seems that between two bad options, the best option is moving.

Additionally, informal caregivers were also dependent on the care approach of the dementia case manager. More specifically, the data analysis revealed that case managers adopted either a more guiding role (direct approach)

or a more supportive role (indirect approach). When the approach was more guiding—as seen in Case 2—the case manager considered it her responsibility, together with the family, to make key decisions during the transition process.

Dementia case manager, Case 2: When an informal caregiver tells me: 'I'm no longer able to do this,' then all the alarm bells ring. Then I'm not going to soothe the situation. I take action. That requires a lot of effort from me. You have to dare, have courage... You have to point the way.

Case managers who adopted a more supportive care role (Cases 1, 3, and 4) preferred to present care options to the older persons and informal caregivers, allowing them to make the final decision. This approach was maintained even when the case managers did not fully agree with the decisions that were ultimately made.

Dementia case manager, case 4: Who are we to say... 'Yes, to avoid a crisis you need to fill in these documents and make sure a nursing home becomes available.' I find this very difficult....

The professional caregivers

For professional caregivers, proactively planning the transition was challenging. They were dependent on the preferences and decisions of older people and their informal caregivers. In many cases, older individuals and their families wished to remain at home for as long as possible, which created a barrier to timely planning and anticipation of the eventual move. In Case 4, for example, the dementia case manager recognised the care burden experienced by the informal caregiver and suggested placing the older person on the nursing home waiting list. However, the informal caregiver did not feel mentally prepared to make such a difficult decision at that time.

Dementia case manager, Case 4: I can offer a lot, and I always say: 'I have a lot of possibilities for you to use. However, it is up to you, what you use.'

Finally, the results showed that the condition of older persons often declined suddenly following an illness or incident. This led to a spike in the care burden for informal caregivers, who were already carrying significant responsibilities. Such incidents were often the final tipping point. In response, dementia case managers had to abandon the original care plan and urgently seek an alternative solution—even though they had anticipated the possibility of a sudden decline earlier in the transition process.

Dementia case manager, case 2: The informal caregiver informed me that she could no longer care for her husband. So, when she indicated that to me, I started the transition process.

A crisis-driven rather than a planned transition

This theme highlights the considerable challenges faced by older adults—and especially informal caregivers—in planning the transition in a timely manner. The transition decision is often postponed until it becomes unavoidable, leading to a crisis-driven rather than a planned transition. Factors contributing to this reactive approach include uncertainty around the timing and suddenness of the move, societal taboos surrounding nursing home care, and the influence of government policies.

While on the waiting list for their preferred nursing home, older individuals and informal caregivers began to question their transition decision because of the many uncertainties surrounding the process and the increasing dependency within the care triad. At the start of the study, all older persons and their families had a clear transitional care plan: they had selected a preferred nursing home and were awaiting the availability of a bed. However, during this waiting period, doubts emerged. These stemmed largely from the long and unpredictable waiting times, leaving families caught in a recurring dilemma—whether to delay the transition or act pre-emptively in response to anticipated health decline. Instead of anticipating a potential crisis and proactively planning the move, they began responding reactively to the immediate situation. Specifically, when things were stable and care at home remained manageable, the transition was postponed; but when an unexpected event occurred, the possibility of a nursing home move was suddenly reconsidered. This dilemma frequently surfaced following a health-related incident. Ultimately, the timing of the transition often depended on whether the informal caregiver's responsibilities exceeded what could be managed at home before a nursing home place became available. The quote below illustrates how admission to a nursing home is determined more by availability than by personal choice.

Informal caregiver (wife), case 3: You have to move to a location where a bed is available, and you don't know where that is.... However, you don't have a choice, you have nothing to say. I or we can want something, but that is not how the healthcare system works.

In Case 3, the care burden exceeded what could be managed at home. The older person's health deteriorated, and a crisis occurred, requiring an emergency move to a crisis location within an hour—despite the existence

of a clear transitional care plan. The older person and informal caregiver had taken several preparatory steps: they had visited multiple nursing homes, registered on two waiting lists, and visited their preferred facility several times to ask questions and become familiar with the staff. Nevertheless, a crisis could not be avoided because of the older person's sudden health decline and the lack of available beds at the preferred care location. The situation was highly stressful and emotional for everyone involved. After the older person died, the informal caregiver described the transition process as more distressing than the loss itself.

Informal caregiver (daughter), case 3: It has been a real traumatic experience for my father... Within an hour he had to sit in the car with his bag and he left. He hadn't expected it... It was so sad to see him. He was completely... he felt he was dumped. Like he had done something wrong. It was truly heartbreaking to see.

The dilemma between postponing the transition despite the care burden or addressing it directly could also stem from persistent taboos surrounding dementia and nursing homes. In particular, stigma around a dementia diagnosis remains strong. Older adults with dementia often internalise this stigma and strive to appear healthy for as long as possible. As a result, friends and family of participants sometimes made comments to informal caregivers such as, 'He's not that bad,' or 'He seems fine,' which further complicated the decision to move to a nursing home. As the informal caregiver in Case 4 put it: 'A person with dementia surrounded by friends and family is completely different to the older person behind the front door.' These perceptions added to the stress and uncertainty faced by informal caregivers when making the decision to transition. In addition, taboos and negative attitudes toward nursing homes persist. Many people still hope to avoid nursing home admission, often associating it with loss and the end of life. Even one of the case managers in the study expressed a hope that her clients could remain at home and avoid the move altogether. However, it was unclear whether this sentiment was influenced by prior negative experiences with the transition process or dissatisfaction with nursing home care itself.

Dementia case manager, case 4: It sounds very harsh. However, I hope that something happens that will lead to the man's death. This way he does not have to start the journey to a nursing home. I hope those people are spared from that.

Moreover, the phone call from the preferred nursing home informing participants that a place had become

available was always perceived as sudden and unexpected—despite months of waiting and knowing such a call would eventually come. It was nearly impossible to prepare, either practically or mentally, for this moment, which had a significant impact on the lives of both older people and their informal caregivers.

Informal caregiver (wife), case 2: There is a sword of Damocles above my head. Every time the phone rings....

Waiting list coordinator, case 3: It is always remarkable how shocked people are. It always catches people by surprise, no matter how well you tried to prepare them.

Additionally, because of government policies—specifically those related to the limited availability of nursing home beds and the financial incentives tied to occupancy—nursing homes are required to fill a vacant bed within 1 week in order to receive funding. As a result, participants had to make a decision and complete the move within 1 week of receiving the phone call. The suddenness and pressure of this call caused both informal caregivers and older persons to doubt their decision. Dementia case managers often encouraged them to accept the available bed, explaining that refusing it would lower their position on the waiting list and increase the risk of a future crisis. The emotional weight of the phone call and the decision that followed became clear in the days leading up to the move, which were described as highly emotional by all participants. During this time, participants in all cases temporarily withdrew from the study. In Cases 2 and 3, the period was described as a ‘family moment’.

A mostly discontinued transition process

This theme highlights the involvement of multiple professional caregivers throughout the transition, each holding only partial knowledge of the overall care process. This created a sense of interdependency among professionals but also led to a fragmented approach to care. Poor coordination contributed to communication breakdowns at critical moments, particularly during handovers. The absence of a consistently present professional caregiver meant that older persons and their informal caregivers were left without a trusted guide to support them through this complex and emotionally demanding transition.

The interdependency between various healthcare professionals involved in both home and nursing home care—such as the home care nurse, general practitioner, dementia case manager, and nursing home nurse—arose from the involvement of multiple caregivers managing

the same care process. This often left the care triad feeling unsupported and without clear direction. Professionals from different care settings were required to collaborate, yet each was responsible for only a specific aspect of the transition. However, efficient collaboration and leadership were frequently lacking. Professional caregivers relied on information and referrals from others to continue the care process, resulting in fragmented care. The continuity of care also varied depending on the individuals involved; some professionals went the extra mile to support the transition, while others remained more peripheral. When professional caregivers had an established history of working together, their collaboration was noticeably more effective, leading to improved care—an observation echoed by those involved. This is further illustrated by the following quote.

Owner day care firm, Case 3: Interviewer: You probably get more phone calls from certain nursing homes? Participant: Yes, and more significantly, from certain employees.

Moreover, although all dementia case managers initially stated that they would remain involved for up to 6 weeks after the move, none were able to do so because they were assigned new clients as soon as the older person relocated to a nursing home.

Dementia case manager, Case 1: Interviewer: ‘Would you like to play a greater role during the transition?’ Dementia case manager: ‘I don’t know. Yes, maybe it is needed, but I am so busy. I know I can’t use it as an excuse. However, when you are wrapping up one case... there is a new client that needs visiting...’

Because of the loss of guidance from the dementia case manager, both older people and informal caregivers lost their trusted professional support once the move to the nursing home had taken place. They were left to navigate the administrative procedures, routines, and expectations of the nursing home without assistance. This was particularly evident in Case 1, where participants experienced inadequate care and had no access to an independent professional for consultation. Informal caregivers were required to take on additional responsibilities—such as decorating their parent’s room—while simultaneously advocating for their loved ones. They persistently pushed for improvements in care, all while trying to understand and comply with the nursing home’s rules and regulations. For example, one caregiver had to advocate for their loved one to receive non-prescribed treatments, including a basic cough syrup that was not recommended by the home’s geriatric specialist.

Informal caregiver (son), case 3: My mother would have benefitted from some support and aftercare.

The fragmentation of the transitional care process was particularly evident during care handovers, where miscommunication was common. Dementia case managers transferred medical and personal information to the nursing home; however, they were often unsure whether the information had been received and reviewed. They noted that they rarely received any feedback or follow-up questions from the nursing home staff.

Dementia case manager, Case 1: Interviewer: 'Do you have the feeling that the organization uses your care plan?' Dementia case manager: 'No, not at all. I actually do not ever get a response, never.'

Contradictorily, staff from nursing homes reported that they needed information from the home care setting in order to continue the transition effectively. However, they noted that this information was often provided too late or was incomplete. Communication between the two care settings relied solely on written handovers. Professional caregivers from both settings acknowledged the advantages of direct communication. When the researcher inquired about the possibility of verbal handovers, time constraints were cited as a potential barrier. Nonetheless, the caregivers were generally unsure why direct communication between them did not occur.

Staff nursing home, Case 2: Interviewer: 'Would like to have more contact with the dementia case manager or home care professionals?' Professional: 'Yes, I would like that because they have good information regarding the client and we don't have those contacts afterwards... I do not know the reason why we don't keep in contact. I actually don't know.'

Finally, an electronic report detailing the care and daily life of older persons—shared with informal caregivers—positively influenced care continuity. It enabled informal caregivers to anticipate care needs and plan their visits more effectively. Additionally, it supported timely and efficient communication. For instance, the nursing home could inform informal caregivers when an older person was running low on clean laundry, allowing them to bring additional clothing from home. Staff were willing to document and share these reports, recognising their value for informal caregivers. This access kept caregivers informed and allowed them to coordinate their support more deliberately.

Informal caregiver (wife), case 2: If I wake up in the morning, I go to the restroom and then straight to

the computer to see what they are writing. If they write that they are going for a walk, then I can visit tomorrow.

Discussion

In this study, the researchers employed the patient journey mapping methodology to capture the real-time experiences of the care triad over an 18-month period. To our knowledge, this is the first study to document the experiences of the care triad during the transition from home to a nursing home in real time.

The study highlights the pervasive sense of uncontrollability experienced by the care triad during the transition from home to a nursing home, as outcomes often diverged from initial plans and decisions. This sense of uncontrollability stemmed from an experienced dependency within the care triad, a crisis-driven rather than a planned transition, and a largely discontinued transition process. According to participants, these factors contributed to the overall perception that there is no such thing as an optimal transitional care process.

First, our study suggests that the sense of uncontrollability may be closely linked to the experience of dependency within the care triad. It highlights the challenges faced by the triad—particularly the older person—in maintaining open and honest communication. Although the transition process is initially guided by principles of open dialogue and shared decision-making, the involvement of the older person often diminishes as their cognitive abilities decline. This reduced involvement is largely influenced by the uncertainties and concerns experienced by both informal and professional caregivers. However, previous studies have shown that when older individuals are unable to participate in the transition process or make decisions, it hampers their adjustment to the new living environment and undermines their ability to feel at home in the nursing home [11, 32–35]. When not informed, they felt cut off from their previous lives [33]. Responsibility for the transition decision ultimately fell to informal caregivers, even though Kraun et al. (2023) found that these caregivers experienced greater burden when the older person was not involved in the decision-making process [36]. In the post-transition phase, this study also found that older individuals felt a strong sense of dependency on the care provided by nursing home staff. This perceived lack of control may be associated with a reduced quality of life [37], depression [38], and physical decline [37, 39]. In a recent study, Groenvynck et al. (2023) interviewed older persons with dementia about their wishes for future nursing home care, revealing a clear desire to be recognised as autonomous individuals [12]. Failing to inform and include older people in their care process ultimately hindered the fulfilment of their personal wishes and preferences.

Second, the transition decision was more often crisis-driven than planned. Although all older persons and informal caregivers in this study had agreed to a nursing home move, there remained a persistent sense of doubt surrounding the decision. Older people and their informal caregivers struggled to balance the growing need for care with a strong desire to delay the transition for as long as possible. Similar findings were reported by Strang et al. (2006), who observed that informal caregivers experienced a series of crises, each bringing them closer to deciding on a move to a nursing home [40]. Research has shown that informal caregivers often reach a ‘tipping point’—a critical moment that persuades them the move is unavoidable [41, 42]. The current findings build on this by showing that these tipping points are not isolated incidents but part of an ongoing struggle, closely linked to the uncertainty and ambiguity surrounding the transition decision. In particular, long waiting lists for Dutch nursing homes—combined with an urgency-based allocation system—make the timing of the move highly unpredictable. As a result, while older individuals and their informal caregivers may express a preference for a specific nursing home, this choice is often illusory. Ultimately, decisions are shaped more by the older person’s health status and waiting list dynamics than by personal preference. This disconnect between initial expectations and the actual outcome may intensify the sense of lost control among both older people and their informal caregivers. Moreover, this prolonged struggle is influenced by persistent taboos surrounding nursing homes [35]. Research shows that many informal caregivers hold negative perceptions of nursing homes, frequently associating them with death and using terms such as ‘imprisonment’ [43]. These taboos can hinder the transition decision because both older persons and their caregivers may be less willing to accept care when stigmatisation is present [35]. Such stigma is often reinforced by international policies that emphasise ageing in place, promoting the idea that people should remain at home for as long as possible [44]. This policy emphasis can create a mismatch in priorities, with professionals focusing on prolonging home care rather than identifying the most appropriate timing for a nursing home transition. Moreover, while much of the existing research has focused on delaying nursing home moves, there has been comparatively little attention to improving the quality of transitional care itself [42].

Third, this research highlights the influence of professional caregivers’ care approaches on the experiences of both older persons and informal caregivers during the transition process. In particular, the care style of the dementia case manager played a role in shaping the transition decision. In Case 2, for example, the dementia case manager adopted a more guiding approach and successfully encouraged the informal caregiver to proactively

place the older person on the nursing home waiting list. These findings are supported by previous research, which suggests that support and guidance from both professional caregivers and peers are essential and can ease adjustment to the new living environment [45, 46]. Older people and informal caregivers benefit from a partnership with a professional caregiver throughout the transition process [4].

Implications for research and practice

The findings indicate that despite thorough preparation, the transition from home to a nursing home was experienced as a very abrupt move. A clear gap exists between home care and nursing home care, and it is essential that this gap is bridged to ensure continuity of care. To achieve this, changes are needed at the micro, meso, and macro levels.

At the micro level, professional caregivers should work together to enable a more gradual transition process and ensure continuity of care [4]. One potential solution, highlighted in several studies, is the positive role of a dementia case navigator. These trained professionals support and collaborate with older persons living with dementia to help them navigate the healthcare system [47, 48]. Research has demonstrated several benefits of such care navigators, including reduced delays in long-term care transitions [49], improved well-being [50], and enhanced quality of life [51, 52]. Informal caregivers have also reported a reduced burden [51]. In the Netherlands, this role is currently fulfilled by the dementia case manager. However, their position within the care process should be redefined to provide more consistent support and guidance throughout the transitional care journey [24, 53]. In particular, dementia case managers should adopt a more guiding role, encouraging older persons and informal caregivers to make timely transition decisions.

At the meso level, managers should prioritise educating professional caregivers on how to involve older people with dementia in their own care process. This study demonstrated the challenges of informing older individuals about their transition journey, particularly in the context of cognitive decline. Healthcare professionals involved in the study reported that based on their experience, informing older persons about an impending transition was not always perceived as beneficial. Therefore, future research should explore how involving older persons living with dementia in the care process may be beneficial for all parties involved. A recent study showed that education on self-management techniques for older people with dementia can enable them to actively participate in their own care. Policymakers and managers should ensure that staff receive training on the importance of these techniques, as research has shown they

can empower individuals with dementia to engage in their care [54], which in turn may enhance their quality of life [55].

At the macro level, policymakers should collaborate to support a more gradual transition process. An older person's place of residence should not determine the quality of care they receive or the life they are able to lead. Care decisions and provision should be guided by the needs and preferences of the older person, rather than being constrained by their living environment [12]. One way policymakers could facilitate a smoother transition would be to allow older persons to continue attending their day care centre after moving into a nursing home. At present, this is not feasible because of the financial structure of the Dutch healthcare system. Similarly, nursing homes should offer opportunities for older persons and their families to experience life in the facility prior to the actual move; however, this too is currently hindered by financial limitations [56, 57]. Regular visits before the move could help older persons, informal caregivers, and nursing home staff build familiarity and trust. It is clear that the current abrupt nature of the transition process is largely driven by financial and organisational divisions between home care and nursing home care [57]. Each setting has its own routines and practices in care provision and information transfer. A well-developed financial incentive to facilitate continuity of care and a gradual handover might improve transitional care. The financial reimbursement for provided care is then no longer based on the care context but focuses on care coordination [58]. These financial reimbursements could enable dementia case managers to prioritise care coordination and continue serving as the primary point of contact for everyone in the care triad, even after the older person has moved to a nursing home. At present, dementia case managers are funded through the home-care context, and due to their high workload, they feel constrained in their ability to provide effective transitional care.

Finally, to ensure that the care triad experiences a sense of control and is able to take an active role throughout the relocation process, further research is needed to explore the relationship between the three subthemes and to better understand how the care triad can be empowered.

Limitations

Some study limitations need to be acknowledged. First, all of the older persons included in the study received support from both informal caregivers and dementia case managers. However, only 33–41% of people with dementia in the Netherlands receive support from a case manager [24]. The conditions and context at the start of the transition process were therefore relatively favourable. Had participants without dementia case managers and/or informal support been included, the outcomes may have

differed. Nevertheless, even with the additional support of informal caregivers and dementia case managers, the transition processes remained largely uncontrollable and inadequate. Second, the real-time follow-up of participants during such a complex life event presented recruitment challenges. As a result, only three of the five cases fully aligned with the original aim of tracking individuals throughout the entire transition. The remaining two cases captured only part of the process. This is reflective of the prolonged and unpredictable nature of transitional care, which often spans several months or even years, making it difficult to guarantee if or when an older person will move into a nursing home. In addition, researchers were not able to interview all older persons in each care triad because of advanced dementia or the informal caregivers' decision not to involve them. This further illustrates the complexity of the transition process. In cases where the older person had not been informed of the transition decision—common among those close to the move—researchers were ethically unable to conduct interviews. Had all older persons been able to participate, their perspectives could have offered further valuable insight into the care process. Nonetheless, the researcher captured their views as much as possible, also conducting proxy interviews in which informal and professional caregivers were asked to share the older person's perspective when direct participation was not possible [59].

Conclusions

This study demonstrated that care triads experienced an overarching sense of uncontrollability throughout the transition process. This was linked to dependency within the triad, reactive rather than proactive transition decisions, and fragmented care processes. To address these challenges, policymakers and professional caregivers should work together to establish a more gradual and coordinated transition. Professional caregivers should adopt a guiding role throughout the process, ensuring continuity of care and support. Further research is needed to explore the relationship between the three subthemes in order to better understand how to empower care triads during the transition from home to a nursing home.

Supplementary Information

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Supplementary Material 1.

Supplementary Material 2.

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Authors' contributions

LG, BdB, AB, EdV, JPH, TvA, EvR, and HV made substantial contributions to the conception and design of the study. LG collected the data. LG, AB, and EdV analysed and interpreted the data. LG drafted the manuscript. All authors approved the submitted version and have agreed to be personally accountable for their own contributions, ensuring that any questions related to the accuracy or integrity of any part of the work are appropriately investigated and resolved.

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Data availability

The interview data collected to support the findings of this study are available on request from the corresponding author (LG). The data are not publicly available because they contain information that could compromise the privacy of the research participants.

Declarations

Ethics approval and consent to participate

In accordance with the Declaration of Helsinki, ethical approval was obtained from the Medical Ethics Committee of Zuyderland, which confirmed that the rules of the Netherlands' Medical Research Involving Human Subjects Act did not apply (registration number METCZ20210044).

Consent for publication

Not applicable.

Competing interests

The authors declare no competing interests.

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